

Risk Management Plan

HealthFirst Care Improvement Initiative

This Risk Management Plan identifies and assesses the operational, technical, and stakeholder risks facing the HealthFirst Care initiative, prioritizes them using a Likelihood × Impact matrix, and summarizes the project's strengths, weaknesses, opportunities, and threats (SWOT). It draws directly on the Business Requirements Document (BRD), Requirements Traceability Matrix (RTM), Stakeholder Analysis and Engagement Plan, Scope Management Plan/WBS, process models, and dashboard findings produced in the prior labs — every risk and mitigation below is traceable to a specific finding or requirement from that work, not a generic list.

1. Risk Register

15 risks were identified across the three required categories by reviewing patient workflows (process models), resource allocation (dashboard/resource data), and feedback trends (satisfaction data): 5 Operational, 5 Technical, and 5 Stakeholder risks. The full register with mitigation strategies is below and in the companion workbook Risk_Register_Matrix.xlsx (sheet: Risk_Register).

Risk ID	Risk Description	Category	Likelihood	Impact	Severity	Mitigation Strategy
OP-1	Workflow disruption during phased rollout of the new scheduling/check-in system disrupts ongoing patient care.	Operational	Medium	High	High	Follow the phased WBS rollout, with UAT sign-off and a rollback plan required before each phase; increase Leadership/IT engagement during cutover weeks.
OP-2	Clinical and administrative staff resist adopting new automated workflows, reverting to manual habits.	Operational	High	Medium	High	Apply the "manage closely" engagement strategy for Doctors/Nurses/Admin Staff; targeted training and department champions ahead of each rollout phase.
OP-3	Double-booking and scheduling conflicts continue before automated conflict detection (FR1) is fully adopted.	Operational	Medium	Medium	Medium	Prioritize FR1 and FR5 early in the build; track appointment completion rate weekly against OBJ-2 during transition.
OP-4	Resource shortages (nurses, rooms, equipment) during go-live create bottlenecks.	Operational	Medium	High	High	Use the FR3 resource-visibility dashboard to flag shortages proactively; cross-department resource-sharing protocol for the 6 PM peak window.
OP-5	Discharge/billing-clearance handoff delays reduce bed/resource turnover.	Operational	Medium	Medium	Medium	Implement the FR4 automated handoff workflow so Admin Staff see discharge-readiness before requesting billing clearance.
TE-1	A data breach or exploited security vulnerability	Technical	Medium	High	High	Complete NFR3 security remediation before go-live; formal security sign-off as a go/no-go gate at Milestone M5.

	compromises patient data.					
TE-2	System/network downtime disrupts scheduling, notifications, or dashboard availability.	Technical	High	Medium	High	Implement NFR2 reliability requirements and a documented failover plan; monitor uptime as an ongoing KPI.
TE-3	Integration between the scheduling module and patient records system (FR5) fails or creates duplicate data.	Technical	Medium	High	High	Sequence FR5 with dedicated UAT (Milestone M7) before Phase 2 go-live; run parallel reconciliation checks.
TE-4	Legacy data-quality issues (missing IDs, "N/A" placeholders, lookup limitations) mislead decision-making.	Technical	High	Medium	High	Apply documented data-cleaning standards as an ongoing checkpoint; label known lookup limitations on any dashboard that uses them.
TE-5	A reporting assumption (e.g., assumed resource capacity) is mistaken for verified fact by leadership.	Technical	Medium	Low	Low	Require every dashboard assumption to be labeled and confirmed with Leadership before use in a funding/scope decision.
ST-1	Less tech-comfortable patients under-adopt the self-service kiosk/online scheduling.	Stakeholder	Medium	Medium	Medium	Retain the assisted/in-person check-in path alongside the kiosk; offer targeted patient support and training.
ST-2	Hospital Leadership's cost/speed expectations and IT's integration-only scope become misaligned.	Stakeholder	Low	Medium	Low	Route any expectation gap through the documented Scope Change Management Process.
ST-3	Doctors and Nurses do not sufficiently buy into the new handoff workflow (FR4).	Stakeholder	Medium	Medium	Medium	Apply the existing "manage closely" engagement plan during handoff rollout; incorporate UAT feedback before full deployment.
ST-4	A change in Project Sponsor or steering-committee membership reduces Key Player engagement.	Stakeholder	Low	High	Medium	Maintain a documented sponsor/steering-committee continuity note and meeting cadence.
ST-5	Regulatory/data-protection compliance requirements surface late and delay rollout.	Stakeholder	Low	High	Medium	Confirm compliance requirements at project initiation (WBS Phase 1); schedule periodic IT/Legal check-ins.

2. Risk Assessment Matrix

Severity = Likelihood × Impact on a 3-point scale (Low=1, Medium=2, High=3), giving a score of 1–9 mapped to three priority bands: High (score 6–9, red, immediate attention), Medium (score 3–4, yellow, monitor and manage), and Low (score 1–2, green, regular review). Of the 15 risks: 7 are High priority, 6 Medium, and 2 Low.

		IMPACT →		
		Risk Assessment Matrix (Likelihood × Impact)		
		Low	Medium	High
LIKELIHOOD ↑	High	Medium (3) —	High (6) OP-2, TE-2, TE-4	High (9) —
	Medium	Low (2) TE-5	Medium (4) OP-3, OP-5, ST-1, ST-3	High (6) OP-1, OP-4, TE-1, TE-3
	Low	Low (1) —	Low (2) ST-2	Medium (3) ST-4, ST-5
		High — Immediate attention Medium — Monitor and manage Low — Regular review		

Figure 1 — Risk Assessment Matrix (Likelihood × Impact), with risk IDs placed in their scoring cell.

The seven High-priority risks span all three categories (OP-1, OP-2, OP-4, TE-1, TE-2, TE-3, TE-4), confirming that the project's greatest exposure is not confined to one area: patient-care continuity during rollout, staff adoption, resource shortages, security, system reliability, and data integration all carry equal top-priority weight and should be tracked together at each go/no-go milestone gate defined in the WBS.

3. SWOT Analysis

The SWOT below reflects the same evidence base as the risk register — strengths and weaknesses are drawn from what the project's own artifacts show today; opportunities and threats are drawn from how the planned solution and its risks could unfold.

STRENGTHS

- Established Key Player sponsorship: Hospital Leadership and IT engaged weekly per the Stakeholder Engagement Plan
- Traceable requirements framework: every BRD requirement is linked to a business objective and tracked in the RTM
- Cleaned, structured operational data now available across appointments, feedback, and resources for evidence-based decisions
- Defined phased delivery plan (WBS) with milestones and go/no-go gates

WEAKNESSES

- Heavy reliance on manual scheduling, paper check-in, and phone/email coordination — the main source of delay in the process models
- Data silos between scheduling and records systems, with lookup limitations already observed in the cleaned datasets
- Uneven resource utilization (Pediatric high, Orthopedics low) and a General Medicine equipment/room maintenance backlog
- Budget constraint limits the project to integration rather than full legacy system replacement

OPPORTUNITIES

- Automating scheduling and check-in (FR1-FR3) directly targets the largest measured bottleneck (60%+ non-completion, 6 PM peak)
- Centralized task-management/reporting (FR4/FR6) can convert ad hoc communication into an auditable, data-driven process
- Change-management and training alongside rollout can convert current staff/patient resistance into adoption
- Phased rollout allows incremental proof-of-value before full-scale investment, reducing risk to leadership

THREATS

- Legacy security vulnerabilities and network downtime already reported by IT could compromise the new solution if unresolved
- Staff/stakeholder resistance to changing entrenched manual habits could stall adoption regardless of technical readiness
- Regulatory/data-protection requirements not yet fully scoped could delay or constrain rollout
- Broader hospital resourcing pressures could compete with this initiative for funding and attention

Figure 2 — SWOT analysis for the HealthFirst Care Improvement Initiative.

4. Summary & Next Steps

- Treat the 7 High-priority risks (OP-1, OP-2, OP-4, TE-1, TE-2, TE-3, TE-4) as go/no-go criteria at their corresponding WBS milestones (M5 Core Solution Build Complete, M7 UAT Sign-off, M8 Phased Go-Live) rather than tracking them only informally.
- Revisit this register at each phase gate: several Medium risks (OP-3, OP-5, ST-1, ST-3) are transition-period risks that should shrink in likelihood once the corresponding BRD requirement (FR1, FR4) is live, and the register should be updated to reflect that.
- The two Low risks (TE-5, ST-2) need no immediate action but should be reviewed on the same cadence as the rest of the register, per the "regular review" guidance for the Low band.

This Risk Management Plan should be reviewed alongside the Scope Change Management Process: any approved scope change should trigger a re-check of this register, since a change to scope, budget, or timeline can shift a risk's likelihood or impact.